

Lean Canvas

Designed for:

MSS Project – ICT508

Designed by:

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1.0.0

Problem

- Expectant mothers in regional/remote areas who have to travel long distances for specialist maternity services.
- Aboriginal and CALD communities face access and cultural barriers to timely maternal care.
- Pathology, imaging, pharmacy and telehealth services are currently siloed, causing fragmented care coordination
- Manual scheduling and billing processes create administrative burden for hospital staff.

Existing Alternatives

- In-person clinic visits requiring travel
- Ad hoc phone triage
- Generic (non-maternity-specific) telehealth tools.
- Paper-based scheduling and billing

Solution

- Telehealth / remote consultations
- Integrated appointment scheduling
- Billing and claims processing
- Health data analytics and reporting
- Pathology, imaging and pharmacy information exchange all at one place.

Key Metrics

- Number of active registered mothers
- Monthly telehealth consultations delivered
- Average patient travel distance avoided
- On-time appointment rate
- Patient satisfaction score
- Platform uptime

Unique Value Proposition

- One secure platform that brings maternity specialists to expectant mothers wherever they live — reducing travel, closing access gaps, and keeping care teams connected

Assumptions

- Mothers and clinicians will adopt a digital-first workflow
- Rockingham and Mandurah have adequate network infrastructure
- Chosen SaaS vendor can meet HL7/FHIR compliance
- WA Health continues to support and co-fund the initiative

Unfair Advantage

- Existing MSI relationship and track record with WA Health
- Built-in compliance with healthcare data standards (HL7/FHIR) from day one
- Clinical domain expertise embedded via midwife/obstetrician advisors.

Channels

- WA Health referral pathways
- Hospital/clinic administrative staff onboarding
- Pre-go-live awareness campaign for local healthcare providers
- Integration with existing hospital systems

Customer Segments

- Expectant mothers (Perth, Rockingham, Mandurah, incl. remote/Aboriginal/CALD)
- Clinicians (midwives, obstetricians, radiologists, pathologists, pharmacists, telehealth providers)
- Hospital administration staff
- WA Health / WA Country Health Service as strategic partner

Early Adopters

- Digitally comfortable expectant mothers already using telehealth elsewhere
- Perth-based clinicians willing to pilot new workflows
- Hospital administrators seeking efficiency gains
- Regional patients currently underserved

Cost Structure

- Software licensing (~38% of ROM budget)
- Implementation and consulting services (~25%)
- User training and support (~15%)
- Data migration and cleansing (~10%)
- Loan servicing/financing costs (~5%)
- Contingency allowance (~10%)
- Ongoing hosting and third-party integration costs

Revenue Structure

- Initial funding via bank loan (4.8% p.a.) to MSI
- Future subscription/licensing revenue if expanded to other WA regions or interstate
- Potential billing/claims processing revenue share with partner health services
- Projected ROM benefits: \$256K (Yr0) rising to \$1.33M (Yr3)

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